

UUID: F20CA7B6-3109-4A49-9355-5DCED1AD7136
TCGA-FD-A3SP-01A-PR

Redacted

Case#:

Sample#

Gender: Male

DOB:

Race:

Report Date:

Pathology Report:

Criteria	Yes	No
Diagnosis Discrepancy	☒	☒
Primary Tumor Site Discrepancy	☒	☒
IPAA Discrepancy	☒	☒
Prior Malignancy History	☒	☒
Tumor/Syn. Non-unique Primary Noted	☒	☒
Case is (circle):	QUALIFIED	DISQUALIFIED
Reviewed/Analyzed	4/12/12	4/12/12

Please see TCGA Pathologic Discrepancy Form

for

Surgical Pathology Report

ICD-O-3

Carcinoma, urothelial, NOS

8120/3

FINAL PATHOLOGIC DIAGNOSIS

A. Right pelvic lymph nodes; dissection:

- Thirty four lymph nodes, no tumor (0/34).

Site:

path: bladder, NOS

667.9

B. Left pelvic lymph nodes; dissection:

- Fourteen lymph nodes, no tumor (0/14).

CQCF: bladder, wall, posterior

667.4

C. Urinary bladder, prostate and vas deferens; cystoprostatectomy:

- Invasive high grade urothelial carcinoma with focal sarcomatoid dedifferentiation, see pathologic parameters.

- Multifocal urothelial carcinoma in situ involving bladder dome and left lateral wall.

- Right ureteral margin with urothelial carcinoma in situ.

- Prostate with high grade prostatic intraepithelial neoplasia.

4-9-12
KD

D. Left distal ureter; excision:

- Portion of ureter, no tumor.

Urothelial Carcinoma Pathologic Parameters

1. Tumor type: Invasive urothelial carcinoma with focal spindle cell sarcomatoid dedifferentiation (10%).

2. Grade of tumor: High grade.

3. Depth of invasion: Extravesicular soft tissue (macroscopic).

4. Tumor distribution: Multifocal, invasive carcinoma at right lateral wall/dome/posterior wall and urothelial carcinoma in situ at dome, left lateral wall and right ureter.

5. Ureteral margins: Right ureter margin with urothelial carcinoma in situ; no invasive carcinoma seen.

6. Distal urethral margin: Negative for tumor.

7. Soft tissue margin or serosa: Negative for tumor.

8. Lymph nodes: Negative for tumor (0/48).

9. pTNM: pT3b,N0,MX.

Effective _____ this Checklist utilizes the _____ edition TNM staging system for Bladder of the American Joint Committee on Cancer (AJCC) and the International Union Against Cancer (UICC).

xx

[], MD

Interpretation performed by the Attending Pathologist and reviewed with the Resident/Fellow [] M.D.

Electronically Signed Out by [], MD

Clinical History:

The patient is a _____ year-old male with clinical history of malignant neoplasm of bladder, heart unspecified, undergoing radical cystectomy with medial bladder.

Specimens Received:

A: Right pelvic lymph node

B: Left pelvic lymph node

C: Prostate, vas deferens

D: Left distal ureter stitch non margin side

Gross Description:

4 The specimens are received in four containers each labeled with the patient's name and medical record number.

A. The first container is additionally identified as, "1. Right pelvic lymph nodes". Received fresh and placed in formalin is a 6 x 4 x 1.5 cm yellow/brown lobular aggregate of adipose tissue. Sectioning reveals multiple lymph node candidates ranging from 0.2-2.7 cm. Sectioned candidate lymph node reveals normal appearing cut surface. All candidate lymph nodes are submitted as follows:

A1 to A3: 5 candidate lymph nodes in each cassette

A4: 4 candidate lymph nodes

A5: 5 candidate lymph nodes

A6 to A8: 4 candidate lymph nodes in each cassette

A9: 2 candidate lymph nodes

A10: 2 candidate lymph nodes

A11: One candidate lymph node bisected

A12: Section of remaining adipose tissue

B. The second container is additionally identified as, "2. Left pelvic lymph node". Received fresh and placed in formalin is a 6 x 3 x 0.5 cm yellow/brown lobular aggregate of adipose tissue. Sectioning reveals multiple lymph node candidates ranging from 0.2-4.5 cm. The largest candidate lymph node is

sectioned to reveal normal appearing cut surface. All candidate lymph nodes are submitted as follows:

B1 to B2: 3 candidate lymph nodes in each cassette
B3: 5 candidate lymph nodes
B4: 2 candidate lymph nodes
B5-B6: One candidate lymph node, section
B7: Section the remaining adipose tissue

C. The third container is additionally identified as, "3. Prostate, vas deferens".

Received fresh and placed in formalin is a 375 g (post formalin fixation), 17 x 11.5 x 4.5 cm cystoprostatectomy specimen consisting of a 6 x 5 x 4 cm bladder (post formalin fixation) with attached mesenteric fat and a 4.6 x 3.7 x 3.5 cm prostate. The right seminal vesicle measure 3 x 1.8 x 0.7 cm and right vas deferens measures 5.5 x 0.5 cm. The left seminal vesicle measures 3.5 x 1.1 x 0.4 cm and left vas deferens measures 4.5 x 0.5 cm. The right ureteral stump measures 0.5 cm long x 0.4 cm diameter, the left measures 0.6 cm long x 0.4 cm diameter, and both demonstrate intact, patent lumenina.

The right half of the prostate and bladder is inked blue and the left half is inked black. The bladder and prostate are opened anteriorly along the urethra in a Y-shaped incision. The prostate is surrounded by a thin, intact membranous capsule and sectioning demonstrates rubbery, yellow-tan parenchyma with no discrete masses or indurations. Sectioning of the seminal vesicles and vasa deferentia reveal no gross lesion but there fairly extensive adhesion of the left seminal vesicle and vas deferens to the prostate.

The opened bladder reveals a 3.3 x 2.7 cm yellow flat ulcerated lesion located in right lateral wall extending to the dome and posterior wall associated with extensive fibrosis on cut section. Grossly, the mass extends through the muscularis propria and into the perivesicular fat but not to the inked margin (approximately 0.5 cm from ink). The surrounding bladder mucosa is wrinkled, pink-tan with a uniform 0.2 cm wall thickness. Bilateral ureteral orifices, adjacent to the trigone, are identified and probe patent.

Representative sections are submitted as follows:

C1: Distal prostatic urethral margin (apex)
C2: Right ureter resection margin
C3: Left ureter resection margin
C4: Prostate apex
C5-C16: Entire ulceration, from inferior to superior
C17: Uninvolved bladder mucosa bladder dome
C18: Uninvolved bladder mucosa anterior wall
C19: Uninvolved bladder mucosa posterior wall

C20: Uninvolved bladder mucosa left lateral wall
C21-C23: Representative right lobe of prostate from apex to base
C24-C26: Representative left lobe of prostate submitted from apex to base
C27: Uninvolved bladder mucosa trigone
C28: Left seminal vesicle and vas deferens
C29: Right seminal vesicle and vas deferens

D. The fourth container is additionally identified as, "4. Left distal ureter-stitch non margin side ".

Received fresh and placed in formalin is a 1.5 cm segment of ureter with a maximal diameter 0.4 cm (stitch side) and a patent lumen measuring 0.2 cm (stitch side). The non-stitch end of the ureter is sectioned and submitted en face in cassette D1.

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[], M.D.

Pathologic Diagnosis Discrepancy Form

V4.00

Instructions: The Pathologic Diagnosis Discrepancy Form should be completed when the pathologic diagnosis documented on the initial pathology report for a case submitted for Case Quality Control Form completed for the submitted case is inconsistent with the diagnosis provided on the

Tissue Source Site (TSS): _____ TSS Identifier: _____ TSS Unique Patient Identifier: _____

Completed By (Interviewer Name on OpenClinica): _____ Completed Date: _____

Diagnosis Information

#	Data Element	Entry Alternatives	Working Instructions
1	Pathologic Diagnosis Provided on Initial Pathology Report	Invasive high grade urothelial carcinoma with focal sarcomatoid differentiation	Provide the diagnosis/ histologic subtype(s) documented on the initial pathology report for this case. If the histology for this case is mixed, provide all listed subtypes.
2	Histologic features of the sample provided for _____ as reflected on the CQCF.	Invasive urothelial carcinoma (no sarcomatoid differentiation)	Provide the histologic features selected on the Case Quality Control Form completed for this case.

Discrepancy between Pathology Report and Case Quality Control Form

3	Provide the reason for the discrepancy between the pathology report and the Case Quality Control Form.	The sampling of the tissue used for diagnosis was different from the tissue reviewed & sent for submission to _____	Provide a reason describing why the diagnosis on the initial pathology report for this case is not consistent with the diagnosis selected on the Case Quality Control Form.
4	Name of TSS Reviewing Pathologist or Biorepository Director	_____	Provide the name of the pathologist who reviewed this case for _____

I acknowledge that the above information provided by my institution is true and correct and has been quality controlled.

TSS Reviewing Pathologist or Biorepository Director

/Date

I acknowledge that the above information provided by my institution is true and correct and has been quality controlled. The Attending Pathologist or the Department Chairman has been informed or is aware of the above discrepancy in diagnoses.

Principal Investigator Signature

Date